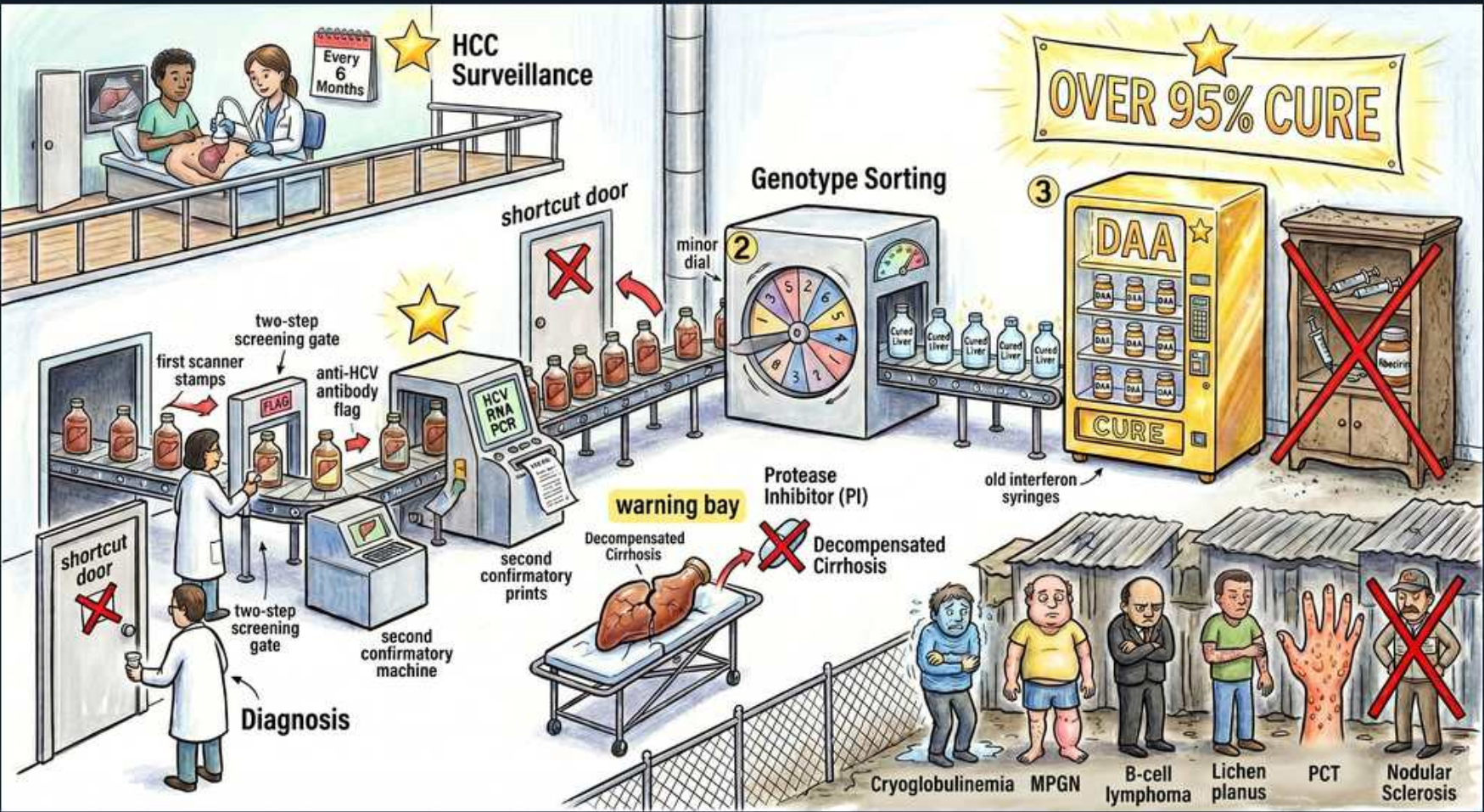


Chronic Hepatitis — HCV





1. Anti-HCV antibody screening gate

WHAT YOU SEE

'first scanner stamps' / 'anti-HCV antibody flag' two-step screening gate, left.

WHAT IT ENCODES

Step 1: screen with anti-HCV antibody (one-time for all adults 18+, plus risk-based). A positive antibody only indicates exposure, not active infection — antibodies persist after spontaneous or treated clearance. Must proceed to confirmatory RNA.

2. HCV RNA PCR confirmation (gold star)

WHAT YOU SEE

'HCV RNA PCR' bottle on the conveyor with a gold star, 'second confirmatory machine', center-left.

WHAT IT ENCODES

GOLD STAR step 2: confirm ACTIVE infection with HCV RNA PCR (quantitative viral load). Positive RNA = active/chronic infection needing treatment; negative RNA after positive antibody = cleared/past infection. This two-step (antibody → RNA) is the core diagnostic algorithm.

3. Positive antibody alone (shortcut door, red X)

WHAT YOU SEE

'shortcut door' with a red X near the screening gate, far left.

WHAT IT ENCODES

A positive anti-HCV antibody ALONE does NOT equal active infection — you cannot skip the RNA step. Treating or labeling chronic HCV off antibody alone is the classic error; cleared patients keep positive antibodies for life.

BOARD TRAP

'Positive anti-HCV antibody means active infection / start treatment' → wrong; antibody only shows exposure — confirm with HCV RNA PCR first.

4. Genotype sorting wheel

WHAT YOU SEE

Roulette-style 'Genotype Sorting' machine, center.

WHAT IT ENCODES

Determine genotype (1–6) — historically guided regimen/duration. Modern pangenotypic DAAs (e.g., glecaprevir/pibrentasvir, sofosbuvir/velpatasvir) reduce the need, but genotype/resistance still matters in cirrhosis or prior treatment failure.

5. DAA vending machine — >95% cure (gold star)

WHAT YOU SEE

Yellow 'DAA CURE' vending machine under an 'OVER 95% CURE' banner with stars, top right.

WHAT IT ENCODES

GOLD STAR: direct-acting antivirals (DAAs) cure >95% with 8–12-week oral regimens, no interferon. Sustained virologic response at 12 weeks (SVR12) = cure. Pangenotypic combos are first-line; check drug interactions (e.g., amiodarone + sofosbuvir → bradycardia).

6. Old interferon syringes (red X)

WHAT YOU SEE

Cabinet of 'old interferon syringes' with a big red X, far right.

WHAT IT ENCODES

Interferon/ribavirin-based therapy is OBSOLETE for HCV — poorly tolerated, lower cure. All-oral DAAs replaced it. Choosing PEG-IFN/ribavirin for modern HCV is a wrong, outdated answer.

BOARD TRAP

'Treat HCV with PEG-interferon (± ribavirin)' → wrong; interferon-based therapy is obsolete — use all-oral DAAs (>95% cure).

7. Protease inhibitor — decompensated cirrhosis (red X)

WHAT YOU SEE

Patient on a stretcher in the 'warning bay', 'Protease Inhibitor (PI) — Decompensated Cirrhosis' with red X, center-bottom.

WHAT IT ENCODES

Protease-inhibitor-containing DAAs (the '-previr' drugs: glecaprevir, voxilaprevir, grazoprevir) are CONTRAINDICATED in DECOMPENSATED (Child-Pugh B/C) cirrhosis — risk of hepatic decompensation/failure. Use a PI-free regimen (sofosbuvir/velpatasvir ± ribavirin) there.

BOARD TRAP

'Give a protease-inhibitor DAA (-previr) to a decompensated Child-Pugh C cirrhotic' → wrong; PIs are contraindicated in decompensated cirrhosis — use a PI-free regimen.

8. HCC surveillance every 6 months

WHAT YOU SEE

Ultrasound exam with 'Every 6 Months' calendar, 'HCC Surveillance', top left.

WHAT IT ENCODES

Cirrhotic HCV patients need HCC surveillance every 6 months (ultrasound ± AFP) EVEN AFTER cure — SVR lowers but does not eliminate HCC risk once cirrhosis is established. Non-cirrhotic cured patients do not need ongoing surveillance.

9. Cryoglobulinemia / MPGN

WHAT YOU SEE

Figures labeled 'Cryoglobulinemia' and 'MPGN', bottom right row.

WHAT IT ENCODES

Extrahepatic HCV: mixed (type II) cryoglobulinemia → palpable purpura, arthralgia, neuropathy; and membranoproliferative glomerulonephritis (MPGN). Both are immune-complex mediated; treating HCV often improves them.

10. B-cell lymphoma

WHAT YOU SEE

Suited figure labeled 'B-cell lymphoma', bottom right.

WHAT IT ENCODES

HCV is linked to B-cell non-Hodgkin lymphoma (often marginal-zone), via chronic antigen stimulation — antiviral therapy can induce lymphoma regression.

11. Lichen planus / PCT

WHAT YOU SEE

Figures 'Lichen planus' and a hand 'PCT' (porphyria cutanea tarda), bottom right.

WHAT IT ENCODES

Skin associations: lichen planus (purple polygonal pruritic papules, Wickham striae) and porphyria cutanea tarda (photosensitive blistering, fragile dorsal-hand skin). Both are recognized HCV extrahepatic manifestations.

12. Nodular sclerosis distractor (red X)

WHAT YOU SEE

Figure 'Nodular Sclerosis' with a red X, far bottom right.

WHAT IT ENCODES

Nodular sclerosis is a HODGKIN lymphoma subtype and is NOT associated with HCV. HCV links to B-cell NON-Hodgkin lymphoma — nodular sclerosis is a pure distractor among the extrahepatic associations.

BOARD TRAP

'Nodular sclerosis (Hodgkin) is an HCV association' → wrong; HCV is tied to B-cell NON-Hodgkin lymphoma, not nodular-sclerosis Hodgkin.